

JOHNS HOPKINS MEDICINE – NEUROLOGY REFERRAL (FAX RECEIVED)

Fax timestamp: 03/14/2026 14:22 EST

Referring office: NOVA PRIMARY CARE ASSOCIATES

Phone: (703) 555-1842 Fax: (703) 555-1991

Referral reason: headache eval / ?migraine vs chronic tension vs other

Requested clinic: NEUROLOGY (general)

Priority: routine (pt states worsening)

Notes: pt w/ daily HA, failed multiple meds, please evaluate

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PATIENT DEMOGRAPHICS

Printed: 03/14/2026 13:58

Name: Carter, Melissa A

DOB: 08/11/1984

Age: 41

Sex: F

MRN (external): NPCA-882193

Address: 14209 Willow Bend Ct, Centreville VA 20121

Phone: (571) 555-0042 (cell)

Email: mcarter84@gmail.com

Emergency contact:

James Carter (spouse) (571) 555-9931

Insurance:

CareFirst BCBS PPO

Member ID: XJH00922311

Group: 77-VA-902

PCP: R. Singh, MD

Allergies:

NKDA (note: later note says nausea w/ codeine?)

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REFERRING PROVIDER NOTE

Date: 03/12/2026

Provider: R. Singh, MD

Chief Complaint: “headaches every day”

HPI:

41 y/o female w/ long hx headaches now reporting daily headaches for “months to years.” States HA started in teens, worsened after second pregnancy (2016). Currently reports headaches “basically every day” though variable intensity. Some days mild pressure, other days severe throbbing. Often bilateral but sometimes R sided.

Associated sx: photophobia sometimes, nausea occasionally, denies vomiting (prior notes say vomiting present). Reports neck tightness. Occasionally wakes up w/ HA.

Tried OTC ibuprofen, acetaminophen almost daily. Previously prescribed sumatriptan (pt says helps “sometimes”). Also tried propranolol in past (unclear adherence).

Denies focal neuro deficits. No vision loss but reports “blur sometimes.”

Assessment: chronic headache disorder, possible chronic migraine vs tension vs medication overuse

Plan:

Refer neurology

Encourage reduction OTC analgesics

Continue sumatriptan PRN

Consider topiramate (pt hesitant)

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PRIOR NOTE #1

Date: 01/08/2026

Provider: R. Singh, MD

CC: follow up headaches

HPI:

Pt w/ headaches since teenage years, worsening frequency. Now ~20 days/month per pt (today says daily). Pain described as throbbing, sometimes pressure. Location varies. +photophobia, +phonophobia (not mentioned in latest note). +nausea, rare vomiting.

Triggers: stress, poor sleep, certain foods (chocolate?)

Medications:

Ibuprofen daily (600–800 mg)

Sumatriptan PRN (uses 2–3x/week)

Exam: nonfocal

Assessment: migraine headaches, increasing frequency

Plan:

Start propranolol 20 mg BID (unclear if pt started)

Limit NSAIDs

Hydration

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PRIOR NOTE #2

Date: 09/22/2025

Provider: Urgent Care (illegible signature)

CC: severe headache

HPI:

Pt presents w/ 3 day hx severe headache, described as “worst in months.” Located occipital region radiating forward. Associated w/ nausea and vomiting x1. Reports similar headaches intermittently for years.

Denies trauma. Denies fever.

Exam:

BP 148/92

Neuro grossly intact

Treatment:

Ketorolac IM

Ondansetron

Discharge dx: tension headache vs migraine

Instructions: follow up PCP

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PRIOR NOTE #3

Date: 06/03/2025

Provider: GYN clinic

Reason: annual exam

Incidental note:

Pt reports “frequent headaches” esp around menses. Takes OTC meds frequently.

PMH copied forward:

Headaches

Anxiety

GERD

Seasonal allergies

Assessment:

Headaches – follow w/ PCP

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LAB RESULTS

Collected: 02/10/2026

CBC:

WBC 6.2

Hgb 12.8

Plt 310

CMP:

Na 139

K 4.1

Cr 0.8

AST 19

ALT 21

TSH: 2.1

Vitamin D: 24 (low)

B12: 410

Lipid panel (irrelevant):

Total chol 212

LDL 134

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IMAGING REPORT

MRI BRAIN W/O CONTRAST

Date: 11/18/2025

Indication: chronic headaches

Findings:

No acute intracranial abnormality.

No mass, hemorrhage, or infarct.

Mild nonspecific white matter hyperintensities, likely chronic microvascular vs migraine-related (clinical correlation advised).

Impression:
Unremarkable MRI brain

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MEDICATION LIST (EHR EXPORT)
Printed: 03/14/2026

Active meds:
Ibuprofen 200 mg tabs – takes 3–4 tabs daily PRN
Acetaminophen 500 mg – PRN (frequent)
Sumatriptan 50 mg – PRN
Propranolol 20 mg BID (pt unsure if still taking)
Omeprazole 20 mg daily
Sertraline 50 mg daily
Loratadine 10 mg daily
Vitamin D3 1000 IU daily
Magnesium supplement (OTC)

Old meds (still listed):
Topiramate 25 mg (never started?)
Cyclobenzaprine PRN
Codeine/APAP (allergy? nausea noted elsewhere)

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DISCHARGE SUMMARY (URGENT CARE)
Date: 09/22/2025

Diagnosis: headache

Course:
Pt treated w/ IM ketorolac w/ improvement. Observed 45 min. Discharged stable.

Instructions:
Return if worsening
Follow up PCP

Medications given: ketorolac, ondansetron

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PROCEDURE NOTE
Date: 09/22/2025

Procedure: IM injection
Medication: ketorolac 30 mg IM
Site: L deltoid
Tolerance: good

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ADDITIONAL NOTE (PARTIAL / TRUNCATED)

Date: 12/02/2025

...patient reports headaches nearly daily, worse in AM sometimes...
...taking OTC meds "almost every day"...
...sleep poor, 5-6 hrs/night...

Plan:

...consider neurology referral if no improvement...

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PAST MEDICAL HISTORY (copied forward multiple notes)

Headaches
Anxiety
GERD
Allergic rhinitis
Vitamin D deficiency

SURGICAL HISTORY

C-section x2

FAMILY HISTORY

Mother: headaches (type unclear)
Father: HTN

SOCIAL HISTORY

Non-smoker
Occasional alcohol
Works desk job

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REVIEW OF SYSTEMS (from 01/08/2026 note copied)

Neuro: headaches, no seizures
GI: nausea
MSK: neck pain
Psych: anxiety

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VITALS (various dates)

03/12/2026: BP 126/82 HR 74

01/08/2026: BP 122/80

09/22/2025: BP 148/92

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FAX COVER ADDENDUM

“Patient calling frequently about headaches, feels not controlled”

“Would appreciate eval for alternative therapies”

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END OF RECORD

Transmission complete

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